

Pregnancy Optimisation Plan

Personalised plan for a couple in Kwinana, Perth WA: female 28, male 33, trying for 2–3 years, no previous pregnancies

Prepared 22 September 2026. Evidence-based education to take to your GP, not a substitute for individual medical care. Prices and service details were checked online in September 2026 wherever a source is cited. Anything marked **(unverified)** or "approx." must be confirmed by phone before you rely on it. Citations in [square brackets] refer to the numbered source list at the end.

Diagnosis-level problem

More than 12 months without conception meets the clinical definition of infertility. You need investigation, not optimisation.

Biggest gaps

No semen analysis ever. Heavy, painful periods never investigated. Sex once a week.

Cost of the first step

\$0 to about \$150 for the core workup, using a bulk-billing GP and Medicare-rebated tests.

0. The honest summary: read this first

You are not a "let's optimise diet and gadgets" couple. You are a "get a proper infertility workup this month" couple.

About 84% of couples conceive within 12 months of regular unprotected sex, and about 92% within 2 years [34]. A 28-year-old woman trying for 2–3 years without a single pregnancy means there is very likely a specific, findable cause. Supplements, food changes, apps and wearables will not fix a blocked tube, endometriosis, an ovulation problem or a low sperm count. Those factors are what you need to find.

Your five most important facts, ranked by how likely each is to matter:

1. **No semen analysis in 2–3 years of trying.** A male factor contributes in roughly 40–50% of infertile couples [35]. The test costs about \$0–\$110 after Medicare. It is the cheapest high-yield test in all of fertility medicine. Book it this week.
2. **Heavy, painful periods.** Together with infertility, these are classic red flags for **endometriosis** or **adenomyosis**. Fibroids and polyps are other possibilities. Endometriosis is found in a large share of women investigated for infertility [26][39]. This needs a proper pelvic ultrasound and probably a gynaecology or fertility specialist opinion. Heavy bleeding also commonly causes **iron deficiency**.
3. **Sex once a week "planned around ovulation".** If the timing is even slightly off (an LH surge missed, tested at the wrong time of day, or sex after ovulation instead of before), a weekly pattern can miss the fertile window in many cycles. The fix is free: sex every 1–2 days through the fertile window, and every 2–3 days the rest of the month [21].
4. **Ovulation has not been confirmed.** A positive LH strip shows the surge, not that an egg was released. A correctly timed mid-luteal progesterone blood test is the cheap confirmation.
5. **Tubal patency is unknown.** Blocked tubes cause no symptoms. If the other tests are normal, a tube test is next. An oil-based contrast tube flush has randomised-trial evidence that it *increases* pregnancy rates in the following six months [22].

About wanting a baby boy: no natural method changes the sex of your baby, and choosing sex through IVF for non-medical reasons is **prohibited in Western Australia and under NHMRC guidelines** [19]. The details are in Section 11. The chance of a boy is about 51% whatever you do. Please don't spend money or cycles on "boy" timing methods. Timing sex late in the window to "favour boys" (the Shettles method) can actually **lower** your chance of pregnancy at all.

About your "normal" tests: "some tests were normal" is not enough information. Ask your GP for printed copies. Standard blood tests often do **not** assess the things most likely to matter here: tubes, endometriosis, confirmed ovulation, and sperm. Section 8 lists what is still needed.

Missing data that matters: actual weights (BMI). At about 168 cm, BMI 25 is about 71 kg and BMI 30 is about 85 kg. BMI over 30 in either partner measurably reduces fertility, and a BMI over 35 excludes you from the public KEMH fertility clinic [1].

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1. Highest-impact actions, ranked by evidence

Every item below is also put into one of four categories: **[A] increases pregnancy/live-birth probability** · **[B] improves biomarkers, with uncertain pregnancy benefit** · **[C] only helps identify ovulation** · **[D] insufficient evidence**.

Tier	Intervention	Class	Why, specific to you
STRONG / HIGH PRIORITY	Full couple infertility workup now: semen analysis, confirmed ovulation, pelvic ultrasound, tubal patency test	A	Finds the cause and gets you onto the right treatment. After 2-3 years this beats every lifestyle change combined [34].
STRONG	Sex every 1-2 days in the 6-day fertile window , and every 2-3 days the rest of the cycle	A	Conception only happens from sex in the 6 days ending on ovulation day [20]. Once a week is too sparse.
STRONG	Specialist assessment of heavy, painful periods (possible endometriosis or adenomyosis)	A	In minimal to mild endometriosis, operative laparoscopy can improve natural pregnancy rates [39]. Moderate to severe disease changes the whole treatment plan.
STRONG	Tubal patency test. If done as HSG, ask about oil-based contrast.	A	Oil-based HSG gave 39.7% vs 29.1% pregnancy within 6 months compared with water-based contrast in a 1,119-woman RCT [22].
STRONG	Folic acid 400-500 µg + iodine 150 µg daily for her	Baby's health	Prevents neural tube defects and protects the baby's brain development [27][28]. It does not raise conception odds, but it is non-negotiable.
STRONG	Healthy weight if either BMI is over 30	A (female), B (male)	Obesity lowers ovulation and pregnancy rates and worsens semen quality. Even 5-10% weight loss helps when overweight.
STRONG	Treat iron deficiency if ferritin is low	Health	Heavy periods make this likely. Fixing it matters for pregnancy health and energy.

Tier	Intervention	Class	Why, specific to you
MODERATE / REASONABLE	LH urine strips to time sex (used correctly)	A (low certainty), C	A Cochrane review suggests timed intercourse with urine LH tests may modestly improve pregnancy rates [33]. Cheap.
MODERATE	Zero alcohol for her; no more than about 1-2 drinks per occasion and few per week for him; caffeine under 200 mg/day	B	You already avoid substances, which is good. Keep caffeine moderate [30][56].
MODERATE	Mediterranean-style diet for both	B	Consistent observational links with better semen quality and fertility outcomes, but no RCT proving more births [40][41].
MODERATE	Avoid testicular heat: no saunas or spas, no laptop on lap, break up long sitting	B	Heat reliably lowers sperm counts, and the effect is reversible over about 3 months.
MODERATE	Vaccinations and preconception checks (rubella, varicella, flu, COVID, cervical screening, genetic carrier screening)	Baby's health	Protects the pregnancy. Carrier screening is Medicare-rebated once per lifetime [37].
LIMITED / OPTIONAL	Omega-3 or a fish-based diet; vitamin D if deficient; general exercise; sleep of 7-9 h	B / D	Good for health. Direct fertility effects are small or unproven.
LIMITED	Male antioxidant supplements (only if the semen analysis is abnormal)	B / D	Large, rigorous trials (MOXI, FAZST) showed no benefit on semen or live births [23][24].
SKIP	Fertility wearables, fertility teas, seed cycling, castor oil packs, detoxes, "fertility massage", crystals, Brazil nuts or pineapple core, sex positions, legs up the wall	D	No human evidence of more pregnancies. Section 11 covers each.
SKIP	AMH as a "can we conceive naturally?" test	—	AMH does not predict natural conception in women your age [31]. It is only useful when planning IVF.
POTENTIALLY HARMFUL	Vaginal steaming, douching (including "baking soda for a boy"), unregulated herbal mixes, testosterone or "T-booster" products for him, megadose vitamin A	—	Testosterone shuts down sperm production. Douching and steaming risk infection and burns. Herbs have unknown effects in early pregnancy.

2. Timing intercourse

The biology in one table

Question	Evidence-based answer
Fertile window	6 days: the 5 days before ovulation plus ovulation day. Conception essentially never follows sex after ovulation [20].
Best days	The 2 days before ovulation and ovulation day carry the highest probability per act of sex (roughly 25–30%+ in fertile couples) [20].
When ovulation happens	About 14 days before the next period (range about 12–16), not "day 14" of every cycle. For a 30-day cycle that is about day 16.
Sperm survival	Up to 5 days in good fertile-type cervical mucus. Most sperm die much sooner.
Egg survival	About 12–24 hours after ovulation. This is why sperm must already be waiting.
Daily vs every second day	Pregnancy rates are very similar. Daily is marginally higher in modelling. Every second day is easier to sustain. Either is far better than weekly [21][34].
How often he should ejaculate	Every 1–3 days. Long abstinence (over about 5–7 days) raises the count but worsens motility and DNA quality. Short abstinence does not "use up" sperm in normal men. Do not "save up" for ovulation day.
LH surge	A positive LH strip (test line as dark as or darker than the control line) means ovulation is likely in about 24–36 hours . Have sex on the day of the positive test and the next day. The day before the surge is also excellent, which is why starting earlier matters.
Cervical mucus	Clear, slippery, stretchy "egg-white" mucus marks the most fertile days, and it is free to track. The last day of slippery mucus ("peak day") is usually within a day of ovulation.
Basal body temperature	It rises about 0.2–0.5 °C after ovulation. It is useful to confirm ovulation happened, in hindsight, but it cannot tell you in advance when to have sex.
Sex position	No evidence any position affects pregnancy rates. Sperm reach the cervix within minutes.
Lying down afterwards	No evidence it helps after sex. It's harmless if you want to do it, but not needed.
Female orgasm	The "upsuck" theory has not been shown to change pregnancy rates in humans. Pleasant, not required.
Lubricant	Many common lubricants reduce sperm motility in lab tests. If you need one, use a "sperm-friendly" one such as Pre-Seed or Conceive Plus, or plain canola or mineral oil. None has been shown to <i>increase</i> pregnancy [25].

Your simplest high-probability schedule

Rule 1 (base layer, whole month): sex **every 2–3 days**, regardless of any test. This alone covers almost every fertile window without tracking [21].

Rule 2 (fertile boost): from about **day 10 of the cycle** (for 28–32-day cycles), switch to **every day or every second day** until 2 days after the positive LH test.

Rule 3: on the day the LH strip turns positive and on the following day, have sex both days.

Rule 4: don't abstain for more than 5 days at any point in the cycle, even outside the fertile window.

How to use LH strips properly (where many people go wrong):

- Start testing about **17 days before your expected period**. For a 28-day cycle that is day 11, for 30 days it is day 13, and for 32 days it is day 15. Start 2 days earlier if your cycles vary.
- Test **twice a day** (about 10 am to midday, and 6–8 pm). Surges can be short, and once-daily testing misses some.
- Reduce fluids for about 2 hours before testing. Don't rely on first-morning urine for most LH strips.
- A **faint line is negative**. Positive means the test line is as dark as or darker than the control line.
- If you never get a clear positive, or you get several positives spread across the cycle, tell the doctor. That can suggest anovulation or PCOS.

Example: 28-day cycle (day 1 = first day of full bleeding)

Day	1-5	6-9	10	11	12	13	14 (LH+)	15	16	17-28
LH test	-	-	-	x2	x2	x2	positive	stop	-	-
Sex	optional	every 2-3 d	✓		✓	✓	✓	✓	optional	every 2-3 d

If the timetable feels like pressure, drop the strips and just have sex every 2 days all month. The pregnancy rate is almost identical and the stress is lower.

3. Fertility devices and technology

Key distinction: almost nothing on this list increases the biological chance of conception. The devices only help you **find** the fertile window. If you already have sex every 2–3 days, their extra value is close to zero. The one "device" that changes outcomes is a **laboratory semen analysis**, because it is diagnostic.

Product	Measures	Evidence	Usefulness	Limitations	Approx. AU cost	Need it?	Best-value choice
Laboratory semen analysis	Volume, count, concentration, motility, morphology, vitality (WHO 2021 method) [36]	Gold standard. Diagnostic.	Essential. Changes management.	Needs 2–7 days of abstinence and delivery to the lab within about 1 h. Results vary, so repeat if abnormal.	\$0–\$110 after Medicare [3][18]	YES, this week	Fertility-clinic andrology lab with a GP referral (Section 9)
LH urine strips (cheap bulk packs)	LH surge	Moderate. May modestly improve pregnancy with timed sex [33].	High value for the price	Can miss short surges. False readings in PCOS.	\$15–\$35 for 40–50 strips	Yes (you already have them)	Generic strips, tested twice daily
Digital ovulation tests (e.g. Clearblue Digital / Advanced)	LH (plus oestrogen in the dual-hormone version)	Same principle as strips	Easier to read. The dual-hormone version flags a wider window.	About 5–10× the cost per test. Same false-reading issues.	\$35–\$70 for 10–20 tests	Optional	Only if you misread strip lines
Clearblue-style fertility monitor	Oestrogen metabolite + LH	Validated for ovulation detection	Some gain over strips	Monitor plus ongoing test sticks	\$200–\$300 + about \$50/month (approx.)	No	—
BBT thermometer	Post-ovulation temperature rise	Confirms ovulation <i>retrospectively</i> only	Low to moderate. Free insight into whether you ovulate.	Disturbed by sleep, alcohol and illness. Cannot predict ovulation.	\$15–\$30	Optional	Use a mid-luteal progesterone blood test instead
Wearable temperature trackers (Oura, Tempdrop, Ava-type)	Skin or overnight temperature, heart rate	Reasonable for <i>retrospective</i> ovulation estimates. No evidence of more pregnancies.	Low for conception	Expensive, and still largely retrospective	Oura about \$450–\$650 + subscription (approx.)	No	—

Product	Measures	Evidence	Usefulness	Limitations	Approx. AU cost	Need it?	Best-value choice
Apple Watch cycle and temperature features	Wrist temperature, used for retrospective ovulation estimates	Manufacturer validation only	Low. Free if you already own one.	Retrospective, not predictive	\$0 if owned	No (fine if owned)	—
Natural Cycles (app + thermometer, ring or watch)	Temperature $\pm$ LH	Regulated as contraception. Evidence for its pregnancy-planning mode is weaker.	Low to moderate	Subscription. Temperature is retrospective.	About \$100–\$200/yr (approx.)	No	—
Inito / Mira / Proov (quantitative hormone readers)	LH, E3G, PdG (progesterone metabolite) $\pm$ FSH	Correlate with lab hormones. No trials showing more pregnancies.	Moderate for tracking. PdG gives some evidence that ovulation happened.	Costly strips. Urine PdG is less reliable than a blood progesterone test.	Reader \$200–\$350 + \$50–\$100/month in strips (approx.)	No	One mid-luteal blood progesterone test does the job for about \$0
Cervical mucus tracking	Mucus quality	Good. Peak mucus closely tracks the fertile window.	High, and free	Takes a couple of cycles to learn	\$0	Yes (free)	Just note "slippery / not slippery" daily
Cycle-tracking apps (calendar-based)	Predicts from past cycle dates	Poor at predicting the day of ovulation	Useful as a diary only	Predictions are often wrong by days	Free to \$80/yr	Free version only	Any free app, used as a log
Sperm-friendly lubricant (Pre-Seed, Conceive Plus)	—	Less harmful to sperm in lab tests. No proven gain in pregnancy [25].	Only if you need lubricant	—	\$20–\$35	Only if dryness	Pre-Seed, or canola oil
At-home sperm tests (e.g. YO, SpermCheck-type)	Motile count or concentration threshold only	Screens for very low counts only	Low	Misses morphology, volume and detail. False reassurance.	\$40–\$120 (approx.)	No	Lab semen analysis instead
Home insemination kits (syringe or cup type)	—	No evidence of benefit for couples who can have intercourse	None for you	—	\$50–\$150	No	—
Cervical-fluid electrolyte devices (e.g. Kegg)	Electrical impedance of cervical fluid	Very limited validation	Low	Expensive	About \$400+ (approx.)	No	—

Product	Measures	Evidence	Usefulness	Limitations	Approx. AU cost	Need it?	Best-value choice
Continuous hormone monitors	—	No consumer continuous hormone monitor is validated and on sale in Australia as of September 2026 (to my knowledge)	—	—	—	No	—

Device prices are typical Australian pharmacy and online retail ranges, not verified quotes. The ranking and advice do not depend on the exact price.

4. Female preconception health

Routinely recommended for every woman trying to conceive in Australia: folic acid (400–500 µg) and iodine (150 µg) daily, no alcohol, no smoking, vaccinations up to date, and a review of any medicines [27][28][30]. **Everything else below is "only if deficient" or "only in specific conditions".**

Topic	Status	What to do (you)
Ovulation	Must be confirmed	Confirm with a blood progesterone test about 7 days before your expected period. If ovulation is irregular or absent, it is treated medically (letrozole is first-line for PCOS), and that works far better than any supplement [43].
Folic acid	Routine	400–500 µg daily, at least 1 month before conception (ideally 3 months) and through the first 12 weeks [28]. You are taking it. Keep going. 5 mg is used when BMI is 30 or more, with diabetes, some epilepsy medicines, or a previous neural tube defect. Ask your GP if BMI is 30 or more.
Iodine	Routine	150 µg daily (NHMRC) [27]. You are currently missing this. Most pregnancy multivitamins include it. Get your GP to check first if you have a thyroid condition.
Iron	Test first	Heavy periods make deficiency likely. Check ferritin. Supplement only if low (typically 60–100 mg elemental iron on alternate days, per your GP).
Vitamin D	Only if deficient or at risk	Medicare restricts routine testing to at-risk people (dark skin, covered clothing, little sun, malabsorption). A pregnancy multivitamin usually contains a small amount, which is fine.
Vitamin B12	Only if deficient or at risk	Test if vegetarian or vegan, on metformin, or if the blood count suggests it.
Omega-3 (DHA/EPA)	Optional	Eat oily low-mercury fish 2–3 times a week. In pregnancy, omega-3 reduces preterm birth in women with low intake [42]. No proven conception benefit.
Choline	Food first	Important for the baby's brain. Most prenats contain little. Eggs (about 150 mg each), meat, fish and legumes cover much of the 440 mg/day pregnancy target.
Calcium	Food first	2–3 serves of dairy or fortified alternatives daily. Supplement only if intake is low.
Zinc	Food first	Present in most prenats. No benefit from extra.
CoQ10	Not routine	Limited evidence, mainly in older women doing IVF with low ovarian reserve. Not relevant at 28.
Inositol	PCOS only	Low-certainty evidence, and the 2023 PCOS guideline does not recommend it as fertility treatment [43]. You have no PCOS diagnosis. Skip it.
Antioxidant stacks	Skip	No evidence of benefit for natural conception.
Prenatal multivitamin	Reasonable	The simplest way to get folic acid + iodine together. Choose one with 400–500 µg folic acid, 150 µg iodine, and no vitamin A above about 800 µg retinol . Generic pharmacy prenats (about \$15–\$30 a month) are as good as premium brands. Elevit has 800 µg folic acid, which is fine but not necessary.
Caffeine	Limit	Under 200 mg/day. That's about 1–2 cafe coffees; energy drinks count [56].
Alcohol	Avoid	NHMRC: no alcohol when planning pregnancy, pregnant or breastfeeding [30].
Smoking and vaping	None	You don't smoke or vape. Also avoid second-hand smoke.

Topic	Status	What to do (you)
Sleep	Healthy habit	7–9 h. Shift work and very short sleep are associated with cycle disruption (observational).
Exercise	Increase moderately	Walking every second day is a good start. Aim for 150 min/week of moderate activity plus 2 strength sessions. Very intense training with weight loss can suppress ovulation. That isn't you.
Weight	Unknown: measure it	At about 168 cm, a healthy BMI range is about 52–70 kg. If BMI is 30 or more, losing 5–10% of body weight improves ovulation and pregnancy outcomes.
Stress	Reduce, but don't blame yourselves	Stress is a real consequence of infertility. Evidence that it <i>causes</i> infertility is weak. Counselling helps you cope and keep going with treatment.
Thyroid	Test TSH	Treat overt hypothyroidism. Subclinical cases are treated by guideline thresholds; your GP or specialist decides.
Vaccinations	Check before pregnancy	Rubella and varicella immunity (blood test). If not immune, vaccinate and wait 28 days before conceiving. Also flu and COVID per current advice. Pertussis is given in each pregnancy at 20–32 weeks.
Dental health	Check-up	Gum disease is associated with pregnancy complications. Get a check-up before pregnancy.
Medicine review	Always	Ask the GP or pharmacist about <i>every</i> medicine and supplement. Concerns include isotretinoin, valproate, some blood pressure medicines (ACE inhibitors, ARBs), methotrexate, warfarin, high-dose vitamin A, many herbal products, and regular NSAIDs around ovulation (can delay follicle rupture).
Cervical screening	Up to date?	Every 5 years from 25. Self-collection is available.
Genetic carrier screening	Recommended to discuss	Cystic fibrosis, SMA and fragile X. A Medicare rebate is available once per lifetime [37].

5. Male fertility: equally important

He has **never had a semen analysis** after 2–3 years of trying. Until that test is done, the whole investigation is incomplete. It is non-invasive and inexpensive, and a result showing severe male factor would change your treatment immediately (for example, from "keep trying" to ICSI).

Understanding the semen analysis (WHO 2021, lower 5th-percentile reference limits [36])

Parameter	Lower reference limit	What it means
Volume	1.4 mL	Low volume may mean an incomplete collection, a duct blockage or retrograde ejaculation.
Concentration	16 million/mL	Very low (under 5 million/mL) triggers hormone and genetic tests.
Total count	39 million per ejaculate	—
Total motility	42%	—
Progressive motility	30%	The most clinically useful single number.
Normal forms (morphology)	4%	Strict criteria. Isolated low morphology is a weak predictor by itself.
Vitality	54% live	—
DNA fragmentation	No WHO cut-off	Not a routine first test [35]. Consider it after recurrent miscarriage or repeated failed IVF.

The "normal" range is a population reference, not a pass/fail fertility test. Men below the limits still father children, and men above them can still have problems. One abnormal result should be **repeated**, ideally about 3 months later (or sooner if the result is severe).

Lifestyle and exposure factors

Factor	Strength	Evidence and action
Testosterone, anabolic steroids, SARMs, "T-boosters"	Major	External testosterone switches off sperm production , sometimes for over a year. Never start it while trying. Tell your doctor if ever used.
Finasteride / dutasteride	Minor to moderate	Can lower semen volume and count in some men, reversibly. Discuss stopping if the semen analysis is abnormal.
Other medicines	Varies	Sulfasalazine, some antidepressants (ejaculation effects), opioids, chemotherapy. Review all medicines with your GP.
Smoking or vaping	Moderate	Lowers semen quality. You don't use either.
Alcohol	Moderate if heavy	Heavy drinking lowers testosterone and semen quality. Light drinking has little effect.
Cannabis	Moderate	Associated with lower counts and motility. Avoid.
Obesity	Moderate	Lowers testosterone and semen quality. Weight loss helps.
Exercise	Positive	Moderate exercise is associated with better semen. Extreme endurance training may reduce it.

Factor	Strength	Evidence and action
Sleep	Small	Very short or very long sleep is associated with poorer semen (observational). Aim for 7–9 h.
Heat: sauna, hot tubs, hot baths	Real	Regular sauna use lowers counts, reversibly over about 3 months. Avoid while trying.
Laptop on lap	Plausible	Raises scrotal temperature measurably. Use a desk. It's free.
Long sitting (both your jobs)	Small	Prolonged sitting raises scrotal temperature a little. Stand or walk for a few minutes every 30–60 min. Low cost, modest evidence.
Tight underwear	Small	A large 2018 study associated boxers with slightly higher concentration than tight briefs. Loose underwear is cheap and harmless.
Cycling	Small	Only very high-volume cycling has been linked with changes. Recreational riding is fine.
Varicocele	Real, if clinical	Found by physical exam. Repair is considered only if it can be felt and semen is abnormal [35]. Don't treat an ultrasound-only finding.
Fever / illness	Real, temporary	A fever over 38.5 °C can depress sperm for 2–3 months. Repeat an abnormal test if there was recent illness.
Occupational exposures	Real in specific jobs	Solvents, pesticides, lead and heavy metals, high heat (bakers, welders, drivers). Kwinana has heavy industry. If either of you works in the industrial strip, tell the doctor what chemicals are involved.

Male supplements: worthwhile or marketing?

Supplement	Verdict	Evidence
Antioxidant combinations (e.g. Menevit-type)	Mostly marketing	The MOXI RCT (vitamin C, vitamin E, selenium, L-carnitine, zinc, folate, lycopene) found no improvement in semen, DNA or live births (15% vs 24% for placebo) [23]. The Cochrane review found very low-certainty evidence, and benefit disappears in the higher-quality studies [32].
Zinc + folic acid	No benefit	The FAZST RCT (2,370 couples, JAMA 2020) found no improvement in semen quality or live births [24].
CoQ10	Uncertain	Small trials show modest changes in motility or count. No convincing live-birth data.
Selenium	Uncertain	Small effects on motility. Excess is toxic. Don't add Brazil nuts on top of a supplement.
Omega-3	Uncertain	Semen biomarker changes in small trials. Fine to get from fish.
Vitamin C / Vitamin E	Skip as separate products	No outcome benefit shown. High-dose vitamin E has safety concerns.
L-carnitine	Uncertain	Some motility data. It was part of the failed MOXI mixture.
Lycopene	Uncertain	Small trials only. Eat tomatoes instead.
Ashwagandha	Weak, with quality concerns	Small trials, mostly from the same research groups. Rare liver injury has been reported. Not recommended.
Men's fertility multivitamins	Poor value	About \$50–\$90 a month with no proven increase in pregnancy. If the semen analysis is <i>abnormal</i> and you want to try something, a plain generic multivitamin (about \$10/month) for 3 months is a reasonable low-cost experiment. Don't expect it to change your treatment plan.

6. Food and fertility

Honest framing: almost all diet-fertility evidence is observational. Healthier eaters also differ in many other ways. Better diets are consistently associated with better semen quality and ovulatory function [40][41]. **No food has been proven to raise pregnancy rates by itself.** So this guide says "associated with" and never "fertility-boosting".

Framework for both of you

Food group	Evidence and practical advice
Mediterranean-style pattern	Vegetables, fruit, legumes, whole grains, nuts, olive oil and fish. This pattern is associated with better semen parameters and IVF outcomes in observational studies. It is the best-supported overall eating pattern.
Fish	2–3 serves a week of lower-mercury fish (salmon, sardines, tinned light tuna, snapper, whiting, barramundi). For pregnancy or planning, FSANZ advises limiting shark/flake, marlin and swordfish to 1 serve per fortnight , and orange roughy and catfish to 1 serve per week , with no other fish in that week [29].
Eggs	An excellent source of choline, protein and B12. Cook fully once pregnant.
Dairy	2–3 serves a day. Some observational data link full-fat dairy with fewer ovulatory problems. The evidence is weak, so choose whichever you prefer.
Whole grains	Associated with better insulin sensitivity. Choose them over refined grains.
Legumes	Plant protein, fibre and folate. Replacing some meat protein with plant protein was associated with less ovulatory infertility in the Nurses' Health Study.
Nuts and seeds	A 2018 RCT (FERTINUTS) found 60 g/day of mixed nuts improved sperm count, vitality, motility and morphology. That is a biomarker result, not a pregnancy result.
Fruit and vegetables	At least 5 serves of vegetables and 2 of fruit a day. Wash well (see pesticides, Section 12).
Olive oil	Use it as your main fat.
Protein	Adequate protein from mixed sources. No "fertility protein" exists.
Red and processed meat	Processed meat (ham, salami, bacon) is associated with poorer semen quality. Keep processed meat occasional and unprocessed red meat moderate (about 2–3 times a week).
Ultra-processed food	Associated with poorer metabolic health and semen quality. Cut down on takeaway meals. Your diet is "mixed outside and home", so aim for 80% home-cooked.
Added sugar and sugary drinks	Sugary drinks are associated with lower fecundability in both partners (PRESTO cohort). Swap for water.
Artificial sweeteners	Weak, inconsistent data. Moderate use is fine.
Caffeine	Under 200 mg/day for her. Moderate intake for him is fine.
Soy	Normal dietary soy (tofu, soy milk) shows no harm to fertility in humans. Scare stories come from animal and supplement megadoses.
Food safety (for pregnancy)	Avoid listeria-risk foods once pregnant: pre-cut salad from bars, soft-serve, raw sprouts, deli meats, pâté, cold smoked seafood, raw eggs, and unpasteurised dairy. Cook meat and eggs fully and reheat leftovers to steaming hot [29].

20 foods worth building meals around

These are associated with better reproductive or pregnancy health through nutrient density, not magic.

1. Salmon / sardines	2. Eggs	3. Lentils	4. Chickpeas	5. Walnuts
6. Almonds	7. Extra virgin olive oil	8. Oats	9. Wholegrain bread / brown rice	10. Leafy greens (spinach, silverbeet)
11. Broccoli	12. Tomatoes	13. Berries	14. Citrus fruit	15. Plain yoghurt
16. Milk / cheese (pasteurised)	17. Lean beef (iron)	18. Chicken	19. Pumpkin seeds (zinc)	20. Iodised salt (small amounts, instead of plain salt)

Foods worth limiting

- Processed meats
- Sugary drinks and energy drinks
- Deep-fried takeaway and trans fats
- Heavily refined snacks and pastries
- High-mercury fish (see FSANZ limits)
- Coffee above about 2 cups a day for her
- Alcohol

Foods with exaggerated fertility claims

- Pineapple core ("implantation")
- Brazil nuts ("lining")
- Pomegranate
- Maca, royal jelly, bee pollen
- "Warming" foods and avoiding cold foods
- Seed cycling
- Alkaline or "fertility" diets
- Full-fat ice cream "for ovulation" (weak signal overhyped)

Simple 7-day example menu

Day	Breakfast	Lunch	Dinner	Snack
Mon	Oats with milk, berries, walnuts	Wholegrain wrap, chicken, salad	Baked salmon, brown rice, broccoli	Yoghurt + fruit
Tue	2 eggs on wholegrain toast, tomato	Leftover salmon + rice salad	Lentil dahl, spinach, brown rice	Handful of almonds
Wed	Yoghurt, muesli (low sugar), banana	Leftover dahl	Lean beef stir-fry with vegetables, noodles	Apple + cheese
Thu	Oats + pumpkin seeds	Tuna (tinned light) and chickpea salad	Chicken, roast vegetables, olive oil	Carrot + hummus
Fri	Vegetable omelette	Leftover chicken sandwich	Homemade pizza on wholemeal base, salad (planned "takeaway night")	Orange + walnuts
Sat	Wholegrain toast, avocado, egg	Minestrone with beans	Grilled barramundi, potatoes, greens	Yoghurt
Sun	Pancakes (wholemeal) + berries + yoghurt	Sardines on toast, salad	Chickpea and vegetable curry, rice	Mixed nuts

Adapt this freely to your cuisine. The pattern is what matters, not these exact dishes. South Asian dals, curries with vegetables, and grilled fish fit the Mediterranean-type pattern perfectly well.

7. Supplements: evidence tables

Safety rules: no megadoses. Avoid **vitamin A (retinol)** above about 3,000 µg/day, which is teratogenic. Avoid herbal fertility blends, because their effects in early pregnancy are unknown. Stop any product your GP hasn't checked once you get a positive pregnancy test. Tell the pharmacist about every supplement.

Female

Supplement	Evidence	Benefit	Dose	Safety	Interactions	Who benefits	Test first?
Folic acid	High	About 70% fewer neural tube defects	400–500 µg (5 mg if at high risk)	Very safe	Some antiepileptics (dose change)	All women	No
Iodine	High (NHMRC)	Baby's brain development	150 µg [27]	Safe	Thyroid disease: ask your doctor	All women	Only if thyroid disease
Iron	High if deficient	Corrects anaemia	Per GP, often 60–100 mg on alternate days	GI upset. Overload if not deficient.	Reduces absorption of thyroxine and calcium (separate doses)	Heavy periods, low ferritin	Yes (ferritin)
Vitamin D	Moderate if deficient	Corrects deficiency. Fertility effect uncertain.	1,000–2,000 IU	Safe at this dose	—	Low sun or deficient	If at risk
Vitamin B12	High if deficient	Prevents anaemia and neural problems	Per GP	Safe	Metformin lowers B12	Vegan or deficient	If at risk
Omega-3	Moderate (pregnancy)	Fewer preterm births in low-intake women [42]	About 500–1,000 mg DHA+EPA	Safe	Anticoagulants	Low fish intake	No
Choline	Low to moderate	Fetal brain	Food first	Safe	—	Low egg or meat intake	No
Calcium	Moderate if intake is low	Bone. Lowers pre-eclampsia risk if intake is low.	Per diet gap	Safe	Iron, thyroxine	Low dairy intake	No
CoQ10	Low	Possibly in IVF poor responders	200–600 mg	Safe	Warfarin	Not you	—
Myo-inositol	Low	PCOS metabolic markers	2–4 g	Safe	—	PCOS only	Needs a PCOS diagnosis
Prenatal multivitamin	Moderate	Covers folate + iodine in one tablet	1 daily	Check vitamin A	—	All women	No

Male

Supplement	Evidence	Benefit	Research dose	Safety	Interactions	Who may benefit	Test first?
Antioxidant combos	Low (negative RCT) [23]	None proven for live birth	Varies	Generally safe	—	Possibly men with abnormal semen (unproven)	Yes: only after an abnormal semen analysis
Zinc	Negative RCT [24]	None	30 mg	Excess causes copper deficiency	Antibiotics	Deficient men only	Yes
Folic acid	Negative RCT [24]	None	5 mg	Safe	—	—	—
Selenium	Low	Motility (small trials)	100–200 µg	Toxic above 400 µg/day	—	—	Yes
CoQ10	Low	Motility biomarkers	200–400 mg	Safe	Warfarin	—	Yes
Omega-3	Low	Biomarkers	1–2 g	Safe	Anticoagulants	Low fish intake	No
Vitamin C	Low	Biomarkers	500–1,000 mg	Safe	—	—	—
Vitamin E	Low	Biomarkers	400 IU	High doses carry bleeding risk	Anticoagulants	—	—
L-carnitine	Low	Motility	2 g	GI upset	Thyroxine	—	Yes
Lycopene	Low	Biomarkers	10–20 mg	Safe	—	—	—
Ashwagandha	Very low	Unclear	675 mg	Liver injury reports	Thyroid, sedatives	—	Not recommended
DHEA / T-boosters / testosterone	—	Harmful	—	Suppress sperm production	—	No one trying to conceive	Avoid

8. Testing before spending money on gadgets

These are the tests that normally make sense for **your situation**: infertility for more than 12 months, her aged 28 with heavy and painful periods, him never tested. Your GP decides the final list. This is what to ask for.

Female

Test	For you?	Timing	Why / notes
Pregnancy test (urine or blood)	If late	Any time	Before any procedure or X-ray.
Full blood count	Yes	Any day	Heavy periods, so check for anaemia.
Iron studies / ferritin	Yes	Any day	Heavy periods.
TSH	Yes	Any day	Thyroid problems affect cycles and pregnancy.
Prolactin	Reasonable	Any day, not just after sex or exercise	Standard in an infertility workup.
Mid-luteal progesterone	Yes, key test	About 7 days before the expected period (day 21 only for a 28-day cycle; day 23 for 30-day cycles)	Confirms ovulation. A mistimed test gives a falsely "low" result, so it may need repeating. Also count the days until the next period.
FSH, LH, oestradiol	Reasonable	Days 2–5 of the cycle	Baseline ovarian function. Helps diagnose PCOS or hypothalamic causes.
AMH	Only if heading to IVF	Any day	Not Medicare-rebated ; usually about \$60–\$100 out of pocket (approx.). It does not predict natural conception [31].
Testosterone / SHBG	Only if indicated	Days 2–5	Only if there is acne, excess hair or irregular cycles (a PCOS screen).
HbA1c / glucose	If BMI is 25+ or other risks	Any day	Diabetes screen.
Vitamin D	Only if at risk	—	Medicare only rebates this for at-risk groups.
B12 / folate	If diet or blood count suggests it	—	—
Rubella IgG, varicella IgG	Yes	Any day	Vaccinate before pregnancy if not immune.
Blood group and antibodies	Yes	Any day	Routine preconception or antenatal test.
STI screen: chlamydia and gonorrhoea PCR, HIV, hepatitis B, hepatitis C, syphilis	Yes (both partners)	Any day	Past chlamydia is a leading cause of tubal damage. Blood-borne virus screening is standard before fertility treatment.
Cervical screening	If due	—	Every 5 years.

Test	For you?	Timing	Why / notes
Reproductive carrier screening (CF, SMA, fragile X)	Recommended	Any day	Medicare-rebated once per lifetime [37].
Transvaginal pelvic ultrasound	Yes, key test	Ideally days 5–10	Checks for fibroids, polyps, adenomyosis, endometriomas and PCO morphology, plus antral follicle count. Ask for an "endometriosis-focused" or deep-endometriosis scan at a practice experienced in this (usually a specialist gynaecological ultrasound practice). A standard scan can miss deep endometriosis [26].
Tubal patency test (HSG or HyCoSy)	Yes , usually ordered by a specialist or fertility GP	Days 6–11 (after bleeding, before ovulation)	An open or blocked tube decides the treatment path. Ask whether oil-based contrast (Lipiodol) is available. It has RCT evidence of higher pregnancy rates afterwards [22][57]. If there is significant endometriosis or tubal disease, the specialist may choose laparoscopy instead.
Laparoscopy	Possibly later	—	This is a specialist decision. Heavy, painful periods plus infertility is a common reason for it. It diagnoses and treats endometriosis and tests the tubes at the same time.

Male

Test	When	Notes
Semen analysis (concentration, motility, morphology, vitality)	Now	2–7 days of abstinence. Collect at the lab or deliver within about 1 hour, kept at body temperature. Use a fertility-clinic andrology lab. PathWest no longer does fertility semen analysis [2].
Repeat semen analysis	If abnormal	Ideally about 3 months later, or 2–4 weeks if the result is severe.
FSH, LH, total testosterone (morning), ± prolactin	If the semen is abnormal, or the history suggests it	Needs to be a morning sample [35].
Karyotype + Y-chromosome microdeletion	If under 5 million/mL or no sperm	Genetic causes of severe male infertility.
CFTR (cystic fibrosis gene)	If no sperm, with low volume or absent vas	—
Scrotal exam ± ultrasound	If the semen is abnormal	Checks for varicocele or testicular problems.
STI and blood-borne virus screen	Yes	Needed before any fertility treatment anyway.
Sperm DNA fragmentation	Not routinely	Only after recurrent miscarriage or repeated IVF failure. Usually about \$150–\$350 with no rebate (approx.).

Tests not worth ordering routinely: AMH "to see if we can conceive naturally" [31]; sperm DNA fragmentation as a first test; routine vitamin D in low-risk people; thrombophilia or "immune" or NK-cell testing (no pregnancy losses so far, and this is poor-value fertility-clinic testing); endometrial receptivity (ERA) tests; hair-mineral or "food intolerance" tests; salivary hormone panels; "hormone mapping" from wellness clinics; home sperm tests in place of a lab test.

9. Perth + Medicare + bulk-billed pathway (lowest cost first)

Ground rules. "Bulk billed" below appears only where a current source states it. Practices change their billing policies. **Always ask "Is this bulk billed for me, and what is the gap?" when you book.** Since 1 November 2025, federal bulk-billing incentives cover **all** Medicare card holders, not just concession holders, and about 81% of GP visits are now bulk billed nationally [14]. Register as a couple for the **Medicare Safety Net** via Services Australia. It matters if you move to specialist treatment [54].

Step 1: Bulk-billing GP, 20-30 minute "long" consult (this week)

Provider	Address	Billing (verified source)	Notes
Wellness Family Doctors	Unit 14, 40 Meares Ave, Kwinana Town Centre WA 6167 · (08) 9419 0800	Listed as bulk billing all Medicare card holders [12]	Closest to you. Book a long appointment for both of you.
Rockingham Medical & Dental Centre (ForHealth)	18 Civic Blvd, Rockingham WA 6168 · (08) 9557 1111	States 100% bulk billing for all GP consults with a Medicare card [13]	About 15 min south.
Other options	HotDoc / Healthengine "bulk billing GP Kwinana" listings [45] [47]	Varies	Filter for bulk billing and for a GP with a women's health interest.
Genea Perth: bulk-billed Fertility GP	Level 2, 190 Cambridge St, Wembley WA 6014 · (08) 9389 4200 [5]	Genea states its fertility assessment with a Fertility GP is bulk billed for all Medicare card holders, including WA, and no referral is needed [4]	About 40 min drive. A good alternative or add-on: a GP who does fertility workups daily, with an andrology lab on site. Individual tests may still carry fees, so ask.

Tip: you can use both routes. Your local bulk-billing GP handles the blood tests and the KEMH referral. Genea's (or another clinic's) bulk-billed fertility GP speeds up the fertility-specific steps: semen analysis, tubal test and specialist triage.

Step 2: Semen analysis (Medicare-rebated with a referral)

Where	Status (verified)	Likely cost
PathWest (public)	No longer offers fertility semen analysis. It only does post-vasectomy samples at QEII Medical Centre [2].	N/A
Fertility-clinic andrology labs: Genea Wembley, Adora Murdoch, Concept/IVFAustralia Subiaco, Fertility North Joondalup, PIVET, Monash, City Fertility	Accept GP referrals. Genea publishes an andrology request form [5]. Medicare item 73523 (schedule fee \$43.85; 85% benefit \$37.30) [3].	Gap usually about \$0-\$110 (IVF Australia quotes about \$150 with about \$43.75 rebated [18]). Call and ask whether it is bulk billed. Low-cost clinics sometimes bulk bill it (unverified).
Australian Clinical Labs (andrology)	Accredited andrology lab with semen drop-off locations, including Adora WA collection sites [17]	Ask. May carry a gap.
Western Diagnostic Pathology	Accepts seminal fluid, but states an out-of-pocket fee for fertility analysis [16]	Gap applies

Where	Status (verified)	Likely cost
Clinipath	No dedicated fertility semen page found. Check before going.	Unverified

Closest practical option from Kwinana: Adora Fertility Murdoch (Tower C, Suite 307, Level 3, 44 Barry Marshall Pde, Murdoch WA 6150; (08) 6146 5100) [6]. That is about 20–25 minutes' drive, which suits the 1-hour delivery rule if collecting at home. Adora describes itself as a bulk-billing clinic [6][7]. Confirm whether semen analysis specifically is bulk billed.

Step 3: Blood tests (pathology collection centre)

Clinipath (Sonic) states that it bulk bills **routine** Medicare-eligible pathology for eligible patients, but not complex tests [15]. WDP and Australian Clinical Labs have their own policies; check before going. Routine tests usually bulk billed when requested by a GP with a clinical indication include FBC, iron studies, TSH, progesterone, FSH, LH, oestradiol, prolactin, rubella and varicella serology, the STI screen and HbA1c. **Not rebated:** AMH. Both Clinipath and WDP have collection centres across the Kwinana–Rockingham area; use their online locators.

Step 4: Pelvic ultrasound (Medicare-rebated)

A transvaginal pelvic ultrasound has a Medicare item when referred by a GP. Bulk billing varies by radiology provider. Expect \$0 to about \$150 gap at a general radiology practice, and about \$150–\$350 at a specialist gynaecological ultrasound practice for an endometriosis-mapping scan (approx., unverified). Given your symptoms, the specialist scan is usually worth the gap because it answers the endometriosis question much better [26].

Step 5: Referrals, all at once (don't do them one after another)

Service	Details (verified where cited)
KEMH Reproductive Medicine Clinic (public, the only public fertility clinic in WA) 374 Bagot Rd, Subiaco WA 6008 · switchboard (08) 6458 2222 [46]	- Provides publicly funded investigation, cycle tracking, ovulation induction and surgery, plus counselling. It is listed as a "limited" service [1]. Inclusion: woman aged 21–38 at referral, in a relationship, both wanting treatment, unable to conceive naturally [1]. Exclusions: either partner not on Medicare, smoking, illicit drug use, BMI over 35 in either partner, donor sperm needed [1]. Process: the GP emails the referral to kemh.referrals@health.wa.gov.au (subject: "Reproductive Medicine Referral, [Surname]"). KEMH then sends a letter listing prerequisite tests to complete about 60 days before the appointment [1]. Cost: public, so no charge for public outpatient services. Waiting time: no official figure published. One third-party comparison site states 12–24 months for public IVF at KEMH [10] (unverified). Assume it is long. Caution: that same third-party source says eligibility requires <i>no private health insurance</i> [10]. KEMH's own page (as indexed) lists Medicare cover, not insurance status [1]. Ask KEMH before you buy private insurance. - You appear to meet the eligibility criteria, subject to BMI. Get referred now, even if you also go private, so your place in the queue is secure.
Public gynaecology (South Metropolitan Health Service: Fiona Stanley Hospital, Murdoch; Rockingham General Hospital)	Suitable if the ultrasound shows fibroids or polyps, suspected endometriosis or adenomyosis, or heavy menstrual bleeding needing treatment. The GP refers through the WA Health outpatient referral system. Public, so no charge. Waits vary by urgency category (unverified).
Bulk-billing specialist fertility clinic	Adora Fertility (Murdoch and Craigie) describes itself as a bulk-billing specialist-led service from ovulation tracking through to IVF [6][7]. A specialist referral from your GP is needed for the Medicare rebate on the specialist consultation. Ask exactly which items are bulk billed.

Step 6: After the results

- **Everything normal** ("unexplained infertility" after 2–3 years): options are ovarian stimulation with IUI, or IVF. The trying duration of 3 years matters more to prognosis than your ages. Discuss your predicted natural conception chance with the specialist. Oil-based HSG is worth doing first if tubes haven't been tested [22][38].
- **Not ovulating:** ovulation induction with letrozole or clomiphene plus timed intercourse. This is available at KEMH and is low cost privately [1].
- **Endometriosis suspected:** specialist opinion. Laparoscopic surgery or IVF depending on stage [39].
- **Abnormal semen:** repeat test, hormones, urology or andrology review. Severe male factor usually means ICSI.
- **Tubal blockage:** IVF is usually the most effective option.

10. Private fertility options in Perth

Treat this as "if you need treatment," not "go straight to IVF." Figures are from clinic sites or third-party comparisons (September 2026 searches) and change often. Always get a written quote. Medicare rebates reduce specialist and IVF costs, and the **Extended Medicare Safety Net** (2026 threshold \$2,615.50) can raise rebates further once reached [8][53].

Clinic	Location	Initial consult	IVF / ICSI out of pocket (per cycle, after Medicare)	Notes
Adora Fertility	Murdoch (closest to Kwinana); Craigie	Bulk-billing model [6]	About \$2,430 for a fresh IVF/ICSI cycle, excluding medications and anaesthetist [7]	Low-cost, bulk-billing model. Does OI and IUI too.
Genea Perth	Wembley	Bulk-billed Fertility GP assessment [4]	Specialist IVF pricing on Genea WA cost page [55]	Also a Mandurah satellite (unverified).
Fertility North	Joondalup	\$290 fee, \$77 rebate, \$213 gap [8]	About \$6,600 (fee about \$9,500 less about \$2,900 rebate) [8]	Prices updated 1 July 2026 [8]. Far from Kwinana.
Monash IVF Perth	Perth metro	—	About \$7,200 . "Access" program about \$4,800 for eligible patients [11]	Third-party figures [11]
PIVET Medical Centre	Leederville	—	About \$4,000–\$7,000 [10]	Third-party figures
Fertility Specialists of WA	Claremont	Private billing, no bulk billing [9]	See their 2026 fee schedule [9]	—
Concept Fertility (now IVFAustralia)	Subiaco; Joondalup	Bulk-billed fertility GP (third-party) [10]	See fee schedule	—
City Fertility Perth	Perth	—	See cost page	—

Service	Typical Perth out-of-pocket (approx., unverified ranges)	Comment
Specialist initial consult	\$0 (bulk-billing clinics) to about \$250	A GP referral is needed for the rebate.
Semen analysis	\$0–\$110	Item 73523 [3]
Pelvic ultrasound	\$0–\$350	Depends on scan type and provider
HSG / HyCoSy	About \$150–\$500 (unverified)	Ask about oil-based contrast
Ovulation induction (tablets + tracking)	About \$100–\$800 per cycle (unverified)	Cheap and effective when you don't ovulate
IUI (± stimulation)	About \$500–\$2,000 per cycle (unverified)	An option for unexplained infertility [38]
IVF / ICSI	About \$2,430 (Adora) to \$7,200+ (full-fee clinics) per cycle [7][8][11]	Plus medications, anaesthetist and day hospital at some clinics

Avoid paid "add-ons" unless there is a specific indication: embryo glue, endometrial scratch, time-lapse imaging, IVIG, steroids, "immune" testing, PGT-A for a woman your age, or assisted hatching. Most have no proven live-birth benefit.

11. Myth-busting (human evidence, not testimonials)

Claim	Verdict	Evidence
Pineapple core for implantation	Myth	Based on bromelain. No human fertility study.
Brazil nuts for the uterine lining	Myth	Selenium is fine in food. There is no evidence for the lining, and excess selenium is toxic.
Pomegranate	Myth	No human fertility trials.
Mucinex (guaifenesin) for cervical mucus	Unproven	One tiny uncontrolled 1982 report. Not recommended.
Fertility teas	Myth	No trials. Some herbs (e.g. dong quai, high-dose liquorice) are inappropriate in early pregnancy.
Seed cycling	Myth	No human hormonal or fertility data.
Acupuncture	No benefit	An Australian RCT of 848 IVF patients (JAMA 2018) found no increase in live births [44]. Evidence for natural conception is weaker still. Fine for relaxation if you enjoy it, but not a fertility treatment.
Chinese herbal medicine	Unproven + risk	Low-quality trials. Contamination and interaction risks. Unknown effects in early pregnancy.
Fertility massage / Maya abdominal massage	Myth	No evidence it opens tubes or improves pregnancy.
Castor oil packs	Myth	No evidence.
Legs up after sex	Myth	Sperm reach the cervix within minutes. No evidence after intercourse.
Special sex positions	Myth	No evidence for pregnancy, or for the baby's sex.
Alkaline diets	Myth	Diet does not meaningfully change vaginal or blood pH.
Avoiding cold foods	Myth	A traditional belief with no evidence.
Grounding / earthing	Myth	No evidence.
Red-light therapy	Unproven	No quality human fertility RCTs. Heat near the testes is counterproductive.
Sauna	Harmful for sperm	Lowers sperm counts, reversibly. Avoid while trying. It is also a hyperthermia risk in early pregnancy.
Cold exposure / ice baths	Unproven	No fertility outcome evidence.
Fertility crystals	Myth	No plausible mechanism and no evidence.
Vaginal steaming	Harmful	Documented burns and infection risk. No benefit.
Detoxes / cleanses	Myth	Your liver and kidneys do this. Some products contain laxatives or herbs of concern.
Sperm-friendly lubricants	Partly true	Less harmful to sperm than ordinary lubricants, but not shown to <i>increase</i> pregnancy [25]. Only use one if needed.
Antioxidant stacks	Overhyped	The MOXI and FAZST RCTs were negative [23][24].

Claim	Verdict	Evidence
AMH predicts natural fertility	Myth	A JAMA cohort found low AMH did not reduce natural conception chances in women aged 30–44 [31].

Can you choose a baby boy?

Method	Verdict	Evidence
Shettles method (sex at ovulation for a boy, days before for a girl)	Doesn't work, and can cost you pregnancies	An NEJM study of 625 cycles found timing relative to ovulation has no influence on the baby's sex [20]. Restricting sex to ovulation day also cuts your overall chance of pregnancy.
"Boy" diets (high salt or potassium, cereal for breakfast)	Myth	The 2008 "cereal = boys" study was shown to be a probable false positive from multiple testing [48].
Baking soda douching, pH manipulation	Harmful	No effect on sex. Irritation and infection risk. Can harm sperm survival.
Positions, orgasm, moon calendars, Chinese gender charts	Myth	No evidence.
Sperm sorting / IVF with embryo sex selection	Not legal here for non-medical reasons	Non-medical sex selection is prohibited by NHMRC ART guidelines, and by law in WA, NSW, VIC and SA [19]. It is only permitted to avoid serious sex-linked genetic disease.

Bottom line: about 51% of babies are boys, and nothing you do naturally changes that. Put all your energy into getting pregnant.

12. Environmental exposures: sensible vs fear-based

Exposure	What the evidence shows	Sensible, low-cost step	Fear-based (skip)
Heat (testicular)	Strongest evidence on this list. Reversible drop in sperm.	No sauna or spa. No laptop on lap. Stand or walk every 30–60 min.	Special cooling underwear gadgets
Occupational chemicals	Real risks for solvents, pesticides, lead, and some metals and welding	Follow site PPE. Tell your doctor your job. Change out of work clothes before home.	—
BPA	Observational associations with semen and IVF outcomes. Causation unclear.	Don't microwave food in plastic. Use glass or steel for hot food.	Throwing out every plastic item
Phthalates	Observational links to semen quality and hormones	Less processed or packaged food. Fragrance-free personal products where easy.	Expensive "clean beauty" overhaul
Plastics / microplastics	Found in human tissues, but no human fertility outcome data yet	As above: heat and plastic are the main thing to avoid.	Water filter subscriptions and "microplastic detox"
Pesticides	High occupational exposure is harmful. The dietary-residue effect is uncertain.	Wash produce. Organic is optional.	An all-organic diet as a fertility requirement
PFAS	Associations with some outcomes. Main exposure is contaminated water sites and some products.	Check WA Health advisories if near a contaminated site. Otherwise no action.	Testing your blood for PFAS
Fragrances / cleaning products	Weak evidence	Ventilate, wear gloves, use simple products.	Replacing all products
Non-stick cookware	Modern non-stick pans are low risk if not overheated or badly scratched	Don't heat an empty pan on high. Replace flaking pans.	Throwing out all cookware
Thermal receipts	A BPA/BPS source through the skin, but a small dose	Decline paper receipts. Wash hands.	—
Kwinana industrial area	Localised air-quality concerns exist. No specific fertility data.	Only relevant if you work on-site: use PPE.	Moving house for fertility

13. When to seek medical help

Situation	When to get investigated
Woman under 35, regular cycles, no known problems	After 12 months of trying [34][51]
Woman 35–39	After 6 months
Woman 40+	Straight away
Irregular periods (cycles under 21 or over 35 days) or no periods	Straight away
Painful periods or pain with sex (possible endometriosis), heavy bleeding	Straight away. This applies to you.
Past pelvic infection, chlamydia, or abdominal or pelvic surgery	Straight away
Two or more miscarriages	Straight away (KEMH also assesses recurrent miscarriage [1])
Male: undescended testis, testicular surgery or injury, chemotherapy, erection or ejaculation problems, known abnormal semen	Straight away
You: 2–3 years of trying	Overdue. Start now.

"Do not wait" situations (go to a GP or emergency department the same day): positive pregnancy test with one-sided pelvic pain, shoulder-tip pain, fainting or heavy bleeding (possible ectopic pregnancy). Also go the same day for severe pelvic pain with fever, very heavy bleeding (soaking a pad an hour or passing large clots), a painless testicular lump, or sudden testicular pain (torsion is an emergency).

14. Priority roadmap

THIS WEEK

- ☐ Book a **long (double) bulk-billed GP appointment for both of you**: Wellness Family Doctors Kwinana or Rockingham Medical Centre. Alternatively (or as well), book Genea Perth's bulk-billed Fertility GP, which needs no referral.
- ☐ Take this document plus copies of any previous test results. Ask the GP to request the tests in the "exact test list" below.
- ☐ **Him**: book a semen analysis at a fertility-clinic andrology lab (e.g. Adora Murdoch, Genea Wembley). Abstain 2–7 days beforehand.
- ☐ **Her**: switch folic acid alone to a **pregnancy multivitamin with folic acid 400–500 µg + iodine 150 µg**, or add a separate iodine tablet.
- ☐ Change the sex schedule: **every 2–3 days all month, and every 1–2 days in the fertile window**.
- ☐ Start a simple cycle diary: period start dates, LH results (tested twice daily), slippery mucus days, pain and bleeding heaviness.
- ☐ Weigh yourselves and calculate BMI.
- ☐ Him: no saunas, spas or hot baths, and no laptop on lap. Stand up regularly at work.

NEXT 3 MONTHS (90 days is about one full sperm-production cycle)

When	Medical track	Lifestyle track
Weeks 1–2	GP visit. Referrals written: KEMH, specialist or fertility clinic, ultrasound. Bloods ordered. Semen analysis done.	Start the prenatal vitamin. Start the sex schedule. Cut takeaway meals to 1–2 per week.
Weeks 2–6	Day 2–5 bloods. Mid-luteal progesterone (7 days before period). Pelvic ultrasound on days 5–10. STI screen. Rubella and varicella check.	Walks become 30–40 min on most days, plus 2 strength sessions. Fish twice weekly.
Weeks 4–8	Review results with the GP. Repeat semen analysis if abnormal. Specialist appointment (private bulk-billing clinic, public gynaecology, or both). Tubal test organised.	If BMI is over 25, aim to lose 0.5 kg/week. Keep sleep to 7–9 h.
Weeks 8–12	Treatment plan agreed: ovulation induction, surgery for endometriosis, IUI or IVF, depending on findings. Keep your KEMH referral active.	Keep going. Vaccinate if not immune (then wait 28 days before conceiving).

BUY

Item	Approx. cost
Pregnancy multivitamin with folate + iodine (generic is fine)	\$15–\$30/month
Bulk LH ovulation strips (you already use these)	\$15–\$35 per 40–50
Semen analysis	\$0–\$110
Iron supplement, <i>only if ferritin is low</i>	\$10–\$20/month
Pre-Seed, <i>only if you need lubricant</i>	\$20–\$35

DON'T BUY

Item	Why
Men's fertility antioxidant blends	Negative RCTs [23][24]
Inito, Mira, Oura or similar trackers for conception	No pregnancy benefit
At-home sperm tests	The lab test is better and costs about the same
AMH to "check fertility"	Not predictive for natural conception [31]
Fertility teas, herbs, seed cycling, crystals, steaming	No evidence, some harm

Item	Why
"Gender swaying" kits or diets	Don't work [20]
Acupuncture as fertility treatment	No live-birth benefit [44]
Sperm DNA fragmentation, immune tests	Not indicated now

Top 10 highest-impact actions

1. Semen analysis at an andrology lab this month.
2. A formal infertility workup for her, including confirmed ovulation.
3. An endometriosis-focused pelvic ultrasound, and a specialist review of the heavy, painful periods.
4. Tubal patency testing (ask about oil-based HSG).
5. Sex every 1–2 days in the fertile window and every 2–3 days otherwise. No long abstinence.
6. Referral to KEMH now, plus a bulk-billing specialist clinic in parallel.
7. Folic acid + iodine daily for her.
8. Reach a healthy BMI if either of you is over 30 (or over 25 and rising).
9. Remove testicular heat sources for him and break up sitting.
10. Stop spending on unproven products, and put that money toward diagnostics or treatment.

Top 5 useful purchases

1. Laboratory semen analysis
2. Pregnancy multivitamin (folate + iodine)
3. Bulk LH strips
4. Specialist gynaecological ultrasound (worth any gap)
5. Iron supplement, only if ferritin is low

Top 5 things people waste money on

1. Male "fertility" antioxidant blends
2. Premium hormone-tracking devices and wearables
3. Fertility teas and herbal protocols
4. Acupuncture packages for fertility
5. Unindicated tests (AMH for natural fertility, DNA fragmentation, "immune" panels)

Estimated monthly budget

Level	Monthly	What's included
Minimum	About \$25–\$40	Generic prenatal vitamin + LH strips. All GP visits and pathology bulk billed.
Sensible	About \$40–\$80, plus one-off \$150–\$500 over 3 months	Above, plus iron or vitamin D if deficient, Pre-Seed if needed, and one-off gaps (semen analysis, specialist ultrasound, tubal test).
Premium (not recommended)	About \$150–\$300	Adding a hormone monitor, a wearable and men's supplements. Adds cost, not babies. Put this money toward treatment instead.

Exact list of things to ask your GP

1. "We've been trying for 2–3 years without a pregnancy. Can you start a couple infertility workup today?"
2. "I have heavy, painful periods. Could this be endometriosis or adenomyosis? Can you refer me for an endometriosis-focused transvaginal ultrasound?"
3. "Can you refer [him] for a semen analysis at a fertility-clinic andrology lab? PathWest no longer does them."
4. "Can you time my progesterone test for 7 days before my expected period, not simply day 21?"
5. "Can you check my iron, thyroid, prolactin and immunity, and do STI screening for both of us?"
6. "Should I be on 5 mg folic acid instead of 0.5 mg?" (relevant if BMI is 30+ or other risks)
7. "Can you refer us to KEMH Reproductive Medicine now, and also to a fertility specialist or gynaecologist so we're not waiting?"

8. "Can you arrange reproductive carrier screening?"
9. "Are all of these bulk billed? Which ones will have a gap?"
10. "Please give us printed copies of all our results, including previous ones."

Exact referrals and test requests worth discussing

For her	For him	Referrals
FBC, iron studies, TSH, prolactin; day 2–5 FSH, LH, E2; mid-luteal progesterone; rubella and varicella IgG; chlamydia and gonorrhoea PCR; HIV, hepatitis B, hepatitis C, syphilis; blood group and antibodies; HbA1c (if BMI 25+); carrier screening; transvaginal pelvic ultrasound (endometriosis protocol); cervical screening if due. <i>Later, via specialist:</i> HSG or HyCoSy (oil-based if possible).	Semen analysis (andrology lab, Medicare item 73523); HIV, hepatitis B, hepatitis C, syphilis; chlamydia PCR. <i>If semen is abnormal:</i> repeat, plus FSH, LH, morning testosterone, prolactin; karyotype and Y-microdeletion if under 5 million/mL; scrotal exam or ultrasound; urology or andrology referral.	1. KEMH Reproductive Medicine Clinic (couple referral). 2. Fertility specialist at a bulk-billing clinic (e.g. Adora Murdoch) <i>or</i> Genea bulk-billed Fertility GP. 3. Public gynaecology (Fiona Stanley) if the ultrasound is abnormal or symptoms warrant it.

Perth "bulk-billed first" pathway

1. Bulk-billing GP (Kwinana or Rockingham): consult, pathology and referrals, all \$0 [12][13] → **2.** Bulk-billed routine blood tests at Clinipath (confirm policy) [15] → **3.** Semen analysis at a fertility-clinic andrology lab with a Medicare rebate (ask if bulk billed; otherwise about \$0–\$110) [3] → **4.** Pelvic ultrasound (look for a bulk-billing radiology provider, or pay the gap for an endometriosis-protocol scan) → **5. Parallel referrals:** KEMH (public, free, long wait) [1] *and* Adora Fertility Murdoch (bulk-billing model) [6] *or* Genea's bulk-billed Fertility GP [4] → **6.** Tubal test via the specialist → **7.** Treatment matched to the findings. Go to IVF only if tubes are blocked, there is severe male factor or significant endometriosis, or cheaper treatments fail. If IVF is needed, a bulk-billing clinic costs about \$2,430 per cycle compared with \$6,600–\$7,200 at full-fee clinics [7][8][11].

15. Private health insurance: do you need it?

- **Not for investigations.** GP visits, blood tests, semen analysis, ultrasound and specialist consults are out-of-hospital Medicare services. Private insurance cannot legally pay for out-of-hospital Medicare gaps.
- **For IVF,** insurance only helps with the **in-hospital** part (the egg-collection day-surgery fee and theatre costs). This needs **Gold-tier hospital cover** that includes "assisted reproductive services", with a **12-month waiting period** [50]. A couple's Gold hospital policy in WA typically costs several thousand dollars a year (approx.). That is often more than it saves across 1-2 IVF cycles, especially at a bulk-billing clinic.
- **Pregnancy and birth:** public maternity care in Perth (Fiona Stanley, Rockingham General, KEMH) is free and high quality. Private obstetric care is a comfort choice, not a safety requirement, and also needs a 12-month wait.
- **Lifetime Health Cover loading:** this applies from 1 July after you turn 31 if you don't hold hospital cover. **He is 33**, so he may already attract a loading (2% per year over 30) when he buys cover. She is 28, so there is no loading yet. Check your own situation.
- **Possible KEMH issue:** a third-party source says KEMH public fertility requires no private insurance [10]. Confirm with KEMH first.
- **Verdict:** do not buy insurance for fertility now. Revisit only if the specialist says IVF is likely *and* you've done the maths with a clinic quote, or if you want private obstetric care. If you'd pay the Medicare Levy Surcharge anyway because of your income, basic hospital cover may make sense for tax reasons. That is a separate calculation.

16. Sources

Web sources were searched in September 2026. Government sites (health.wa.gov.au, kemh.health.wa.gov.au, MBS Online) were verified through search-engine indexing of their current pages, because direct page fetches were blocked from the research environment. "Third-party" means a comparison site, not the provider itself.

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Disclaimer: this document is general, evidence-based health information prepared for discussion with your own doctors. It does not replace individual medical assessment. Service details, eligibility and prices change. Confirm with each provider before relying on them.